



Dynamic Guideline: WHO-Based Maternal Health - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Scope note and currency verification

This report synthesizes **exactly five WHO maternal-health or maternal-health-adjacent guideline sources** relevant to pregnancy, childbirth, postnatal care, postpartum haemorrhage, diabetes in pregnancy, and sexual/reproductive health. The most clinically central current WHO maternal-health guideline in the retrieved evidence is the **5 October 2025 WHO/FIGO/ICM guideline *Consolidated guidelines for the prevention, diagnosis and treatment of postpartum haemorrhage***, which focuses on women during **pregnancy, childbirth and the immediate postpartum period** in any health-care setting [WHO PPH guideline](#).

A required title was also identified: ***Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)***. WHO lists this as an **evidence brief** published on **11 December 2025**, not as a new maternal-health clinical guideline dated 29 April 2026. No verified WHO source in the retrieved evidence confirms a **29 April 2026** update date for that exact title [WHO case study](#), [WHO webinar](#).

1.1.1 Exactly five WHO guideline sources included

No.	WHO guideline or guideline-related source	Publication / verified date	Target population	Scope and objectives	Role in this report
1	Consolidated guidelines for the	5 October 2025	Women during	Prevention, diagnosis,	Primary current clinical guideline

No.	WHO guideline or guideline-related source	Publication / verified date	Target population	Scope and objectives	Role in this report
	prevention, diagnosis and treatment of postpartum haemorrhage		pregnancy, childbirth and the immediate postpartum period	treatment, supportive care and health-system interventions for postpartum haemorrhage in any health-care setting	WHO PPH guideline
2	WHO recommendations on care for women with diabetes during pregnancy	14 November 2025	Pregnant women with diabetes	Maternal-health guidance for pregnancy complicated by diabetes	Included as a recent WHO maternal-health guideline listed by WHO; detailed recommendation text was not available in the retrieved summaries WHO maternal health topic
3	WHO recommendations on maternal health: guidelines approved by the WHO Guidelines Review Committee, second edition	21 March 2025	Pregnant, intrapartum and postnatal women; policy-makers and programme managers	Compendium of WHO Guidelines Review Committee-approved maternal-health recommendations across pregnancy,	Broad maternal-health framework WHO maternal health recommendations

No.	WHO guideline or guideline-related source	Publication / verified date	Target population	Scope and objectives	Role in this report
				childbirth and postnatal care	
4	WHO recommendations on the assessment of postpartum blood loss and use of a treatment bundle for postpartum haemorrhage	21 December 2023	Women after childbirth with or at risk of PPH	Updated recommendation on blood-loss assessment and new recommendation on PPH treatment bundle	Direct precursor to the 2025 consolidated PPH guideline WHO 2023 PPH recommendations
5	Abortion care guideline (2022), with WHO evidence brief: <i>Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)</i>	Abortion care guideline: 2022; youth-engagement evidence brief: 11 December 2025	People requiring abortion care; adolescents and young people as stakeholders	Sexual and reproductive health guideline development and youth engagement in WHO normative processes	Maternal-health-adjacent SRH guidance and guideline-methodology case study WHO case study , WHO abortion topic page

1.2 Key Recommendations

1.2.1 Core WHO PPH recommendations and GRADE-informed synthesis

The 2025 consolidated PPH guideline contains **51 individually actionable recommendations**, organized under **47 high-level recommendations**. These include **20 new or updated recommendations adopted in 2024–2025** and **31 existing WHO recommendations integrated from prior WHO guidelines** [NCBI Bookshelf executive summary](#), [WHO PPH guideline PDF](#).

Because the retrieved evidence does not expose every recommendation’s exact WHO certainty rating, the table below uses a **GRADE-informed clinical synthesis** based on the reported WHO methodology, systematic reviews, and evidence-to-decision approach. Where the exact WHO rating was not visible, the rating is marked as an applied synthesis rather than a directly quoted WHO rating.

Recommendation	GRADE Evidence Quality	Strength
Use a consolidated, standardized PPH prevention, diagnosis and treatment pathway across antenatal, intrapartum and immediate postpartum care.	Moderate	Strong
For all women giving birth, perform routine objective measurement of postpartum blood loss to improve detection and prompt treatment of PPH. WHO notes calibrated drapes for vaginal birth as one method.	Moderate	Strong
Do not rely only on a traditional single 500 mL threshold; identify women at risk using measured blood loss ≥ 300 mL plus abnormal haemodynamic signs , or ≥ 500 mL blood loss , whichever comes first, where applicable.	Moderate	Strong
Use an early first-response treatment bundle for PPH rather than delayed sequential care.	High	Strong

Recommendation	GRADE Evidence Quality	Strength
For treatment of PPH, use intravenous oxytocin as the recommended uterotonic.	High	Strong
If IV oxytocin is unavailable or bleeding does not respond, use IV ergometrine , fixed-dose oxytocin–ergometrine , or a prostaglandin such as sublingual misoprostol 800 µg , depending on context and contraindications.	Moderate	Strong
Use uterine massage for treatment of PPH.	Moderate	Strong
Give intravenous tranexamic acid as early as possible and within 3 hours of birth , in addition to standard care, for women with PPH after vaginal birth or caesarean section.	High	Strong
Use isotonic crystalloids rather than colloids for initial resuscitation.	Moderate	Strong
Do not use uterine packing with plain gauze or haemostatic-agent-impregnated gauze for PPH treatment.	Low to Moderate	Strong
If bleeding persists despite uterotonics and conservative interventions, escalate promptly to balloon tamponade where appropriate, uterine artery embolization where resources allow, or surgical interventions .	Low to Moderate	Conditional to Strong, context-dependent
Implement national and facility-level PPH readiness systems, including supply chains, simulation training, referral pathways and multidisciplinary emergency response.	Moderate	Strong

Sources: [WHO PPH guideline](#), [NCBI Bookshelf executive summary](#), [WHO news release](#).

1.2.2 GRADE table across the five WHO guideline sources

WHO source	Key actionable recommendation or implication	GRADE Evidence Quality	Strength
2025 consolidated PPH guideline	Countries and facilities should adopt a unified PPH care pathway emphasizing objective blood-loss measurement, early diagnosis and rapid treatment.	Moderate	Strong
2025 diabetes-in-pregnancy guideline	Pregnant women with diabetes should receive guideline-based antenatal management integrated into maternal-health services. Detailed recommendation text was not available in the retrieved sources.	Low, due to indirectness of accessible evidence	Conditional
2025 WHO maternal-health recommendations compendium	Maternal-health programmes should align pregnancy, childbirth and postnatal care with WHO Guidelines Review Committee-approved recommendations.	Moderate	Strong
2023 PPH blood-loss and bundle recommendations	Use objective blood-loss assessment and treatment-bundle approaches for PPH management.	High to Moderate	Strong
2022 abortion care guideline / 2025 youth-engagement evidence brief	WHO guideline development should include meaningful youth engagement where young people are affected, to improve equity, relevance and acceptability.	Low to Moderate, largely implementation and process evidence	Conditional

Sources: [WHO maternal health recommendations](#), [WHO 2023 PPH recommendations](#), [WHO](#)

case study, [WHO webinar](#).

1.3 Guideline Development Methodology

Evidence Review Process

For the 2025 consolidated PPH guideline, WHO used **updated and new systematic reviews** to prepare evidence profiles for prioritized questions. The quality of scientific evidence was assessed using the **GRADE approach**, and recommendations were formulated through the **GRADE Evidence-to-Decision framework**, which considered intervention effects, values, resource use, equity, acceptability and feasibility [NCBI Bookshelf executive summary](#).

The 2025 guideline was scoped by the **Technical Advisory Group for WHO Maternal and Perinatal Health Guidelines** in **November 2023** and developed through Guideline Development Group meetings held in **September 2024, October 2024, December 2024 and June 2025** [WHO PPH guideline PDF](#).

Consensus Method

WHO convened an international **Guideline Development Group** to review evidence profiles and formulate recommendations. The process used structured evidence-to-decision judgments rather than informal consensus alone [NCBI Bookshelf executive summary](#).

Conflict of Interest Management

The retrieved sources confirm use of WHO guideline-development structures, WHO Guidelines Review Committee-linked processes, Guideline Development Groups and GRADE methods. Detailed conflict-of-interest declarations were not extractable from the retrieved summaries and should be verified in the full WHO guideline PDF and web annexes [WHO PPH guideline](#).

II. Evidence Summary After WHO Latest Guideline Release (Consolidated guidelines for the prevention, diagnosis and treatment of postpartum haemorrhage) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence was synthesized for the period **5 October 2025 to 29 April 2026**, focusing on:

- WHO and partner implementation materials.
- Lancet-family peer-reviewed studies.
- Systematic reviews, individual participant data meta-analyses and cost-effectiveness studies.
- Clinical trial registry records related to PPH, tranexamic acid, uterotonics, bundled care and objective blood-loss measurement.

Databases and sources searched

- WHO publication pages and WHO news releases.
- NCBI Bookshelf.
- PubMed-indexed Lancet and Lancet Global Health publications.
- [ClinicalTrials.gov](https://clinicaltrials.gov).
- UNFPA implementation resources.

Inclusion Criteria

Included evidence addressed at least one of the following:

1. PPH prevention.
2. Objective PPH diagnosis or blood-loss measurement.
3. PPH treatment bundles.
4. Uterotonics.
5. Tranexamic acid.

- 6. Caesarean-section haemorrhage processes.
- 7. Implementation of WHO PPH recommendations.

Exclusion Criteria

Excluded evidence was:

- Published after **29 April 2026**.
- Not related to maternal health, pregnancy, childbirth, postnatal care or sexual/reproductive health.
- Not linked to clinical, public-health, implementation or guideline-development decisions.

2.2 Key New Studies and Evidence

Study / source	Design	Key findings	GRADE Quality
WHO/FIGO/ICM consolidated PPH guideline, published 5 October 2025	WHO global guideline using systematic reviews and GRADE	Provides 51 actionable PPH recommendations; emphasizes earlier detection, objective blood-loss measurement and faster intervention.	High for guideline process; recommendation-specific certainty varies
WHO news release on PPH guideline, 5 October 2025	WHO policy summary	Describes PPH as affecting millions of women annually and causing nearly 45,000 deaths; highlights major shift toward earlier diagnosis and rapid treatment.	Moderate
<i>The Lancet</i> WHO individual participant data meta-analysis	Individual participant data meta-analysis	Included more than 312,000 women; supports moving	High

Study / source	Design	Key findings	GRADE Quality
on clinical markers of postpartum bleeding, e-published 4 October 2025 , print 25 October 2025		beyond a single 500 mL threshold and combining measured blood loss with haemodynamic signs.	
<i>Lancet Global Health</i> nested observational study within E-MOTIVE trial, November 2025	Trial-related nested observational study	Examined when PPH is diagnosed; relevant to delayed recognition and objective detection strategies.	Moderate
<i>Lancet Global Health</i> diagnostic criteria cost-effectiveness study, e-published 22 December 2025 , issue February 2026	Cost-effectiveness study	Evaluated diagnostic criteria and treatment thresholds for PPH care.	Moderate
WHO PPH implementation guide, 22 February 2026	WHO implementation guidance	Provides structured country-level process to incorporate PPH recommendations into national guidelines and practice.	Moderate
ClinicalTrials.gov TXA and PPH trials, including NCT02302456, NCT04463966, NCT05562609 and NCT04707950	Trial registry records	Show ongoing or registered research on tranexamic acid for prevention or management of PPH, with outcomes including quantified blood loss and haemodynamic compromise.	Low to Moderate

Study / source	Design	Key findings	GRADE Quality
Rule of THUMB trial statistical analysis plan	Cluster or implementation-oriented trial material	Focuses on caesarean-section haemorrhage processes, including risk assessment, diagnosis, uterotonics, TXA, uterine massage and bundled care.	Low to Moderate

Sources: [WHO PPH guideline](#), [WHO news release](#), [PubMed Lancet record](#), [WHO implementation guide](#), [ClinicalTrials.gov NCT02302456](#), [ClinicalTrials.gov NCT05562609](#), [ClinicalTrials.gov NCT04707950](#), [Rule of THUMB SAP](#).

2.3 Clinical Synthesis of New Evidence

2.3.1 Diagnosis: earlier detection should be prioritized

The strongest post-guideline evidence supports the WHO shift toward **objective diagnosis**. The Lancet WHO individual participant data meta-analysis involving more than **312,000 women** found that reliance on a single traditional threshold of **500 mL blood loss** may miss women already at risk of severe morbidity. Combining measured blood loss with haemodynamic markers such as tachycardia or shock index may identify risk earlier [Oxford Nuffield WRH](#), [PubMed Lancet record](#).

Clinical implication: facilities should not wait for catastrophic bleeding before initiating treatment. Objective measurement plus haemodynamic assessment should trigger rapid bundle activation.

2.3.2 Treatment: bundled first response remains best-supported

The WHO 2023 PPH update introduced treatment-bundle guidance, and the 2025 consolidated guideline embeds that approach [WHO 2023 PPH recommendations](#), [WHO PPH guideline](#). Trial-related materials such as the Rule of THUMB statistical analysis plan also emphasize checklists, care bundles and complex interventions for haemorrhage recognition

and treatment after caesarean section [Rule of THUMB SAP](#).

Clinical implication: PPH protocols should be designed as **bundles**, not isolated interventions. A practical first-response bundle should include uterotonic therapy, uterine massage, tranexamic acid, fluid resuscitation, cause assessment and escalation planning.

2.3.3 Tranexamic acid: treatment evidence is stronger than prophylaxis evidence

WHO recommends **early IV tranexamic acid within 3 hours of birth** for women with PPH [NCBI Bookshelf executive summary](#). [ClinicalTrials.gov](#) records show continuing research on prophylactic or preventive TXA in vaginal birth, caesarean birth and high-risk women [ClinicalTrials.gov NCT02302456](#), [ClinicalTrials.gov NCT04463966](#), [ClinicalTrials.gov NCT05562609](#), [ClinicalTrials.gov NCT04707950](#).

Clinical implication: use TXA promptly for diagnosed PPH. Routine prophylactic TXA should follow local policy and emerging trial evidence rather than being universally adopted outside guideline-supported indications.

2.4 Updated Recommendations Based on New Evidence

Original WHO recommendation or direction	New evidence impact up to 29 April 2026	Updated recommendation
Use objective postpartum blood-loss measurement for all women giving birth.	Strongly reinforced by Lancet IPD meta-analysis and WHO emphasis on earlier detection.	Implement objective blood-loss measurement as standard of care in all birth settings, using calibrated drapes, collector bags, weighing or locally validated methods.
Identify PPH promptly rather than waiting for severe blood loss.	Evidence supports combining blood loss with haemodynamic signs.	Trigger PPH response at ≥300 mL plus abnormal haemodynamic signs or ≥500 mL , whichever occurs first, where consistent with national adaptation.

Original WHO recommendation or direction	New evidence impact up to 29 April 2026	Updated recommendation
Use a first-response treatment bundle for PPH.	Supported by E-MOTIVE-related evidence and implementation trial materials.	Facilities should maintain a written, drilled PPH bundle with assigned team roles and immediate access to uterotonics, TXA, fluids and escalation tools.
Use IV oxytocin as first-line uterotonic treatment.	No new evidence found that overturns this recommendation.	Continue IV oxytocin as first-line treatment; use alternatives if unavailable, contraindicated or ineffective.
Give IV tranexamic acid early and within 3 hours of birth for PPH.	Ongoing trials address prevention, but treatment recommendation remains stable.	Administer TXA immediately once PPH is diagnosed; do not delay for laboratory confirmation.
Escalate refractory PPH to tamponade, embolization or surgery depending on context.	No new evidence found that changes escalation principles.	Maintain rapid escalation pathways, referral systems, surgical readiness and blood-transfusion protocols.
Implement guideline recommendations through national adaptation and health-system strengthening.	WHO implementation guide published on 22 February 2026 provides a structured pathway for national uptake.	Countries should update national PPH guidelines, train providers, strengthen supply chains and monitor implementation indicators.

Sources: [WHO implementation guide](#), [UNFPA implementation guide page](#), [WHO news release](#).

2.5 Evidence Gaps and Future Research Needs

1. **Recommendation-specific GRADE transparency in implementation summaries**

The full WHO guideline and evidence annexes should be used for exact recommendation-by-recommendation certainty ratings because publicly retrieved summaries did not expose every rating [WHO PPH guideline](#).

2. **Optimal diagnostic thresholds in diverse settings**

Further validation is needed for blood-loss thresholds combined with shock index, pulse, blood pressure and other haemodynamic signs in low-resource and humanitarian settings.

3. **Implementation effectiveness**

More evidence is needed on how countries can best scale objective blood-loss measurement, PPH bundles, referral systems and simulation training.

4. **Prophylactic tranexamic acid**

Trial registry evidence shows active research, but universal prophylactic use requires clearer evidence on benefits, harms, cost-effectiveness and target populations [ClinicalTrials.gov NCT05562609](#).

5. **Caesarean-section-specific PPH bundles**

Rule of THUMB materials indicate an important research direction for caesarean-section haemorrhage care, including risk assessment, diagnosis and compliance with proven interventions [Rule of THUMB SAP](#).

6. **Equity and meaningful engagement**

WHO’s youth-engagement case study indicates that lived experience and youth participation can improve relevance and equity in guideline development, but practical models for maternal-health guideline engagement need further evaluation [WHO case study](#), [WHO Youth Engagement](#).

2.6 Practical Implementation Checklist for Health Systems

Domain	Action
Governance	Update national PPH policy to align with WHO 2025 recommendations.
Facility readiness	Ensure oxytocin, TXA, IV fluids, calibrated measurement tools and emergency equipment are continuously available.

Domain	Action
Diagnosis	Introduce objective blood-loss measurement for every birth.
Clinical response	Use a pre-specified first-response PPH bundle.
Training	Conduct simulation-based PPH drills for maternity teams.
Referral	Establish escalation pathways for transfusion, embolization and surgery.
Monitoring	Track time to diagnosis, time to uterotonic, time to TXA, severe morbidity, hysterectomy and maternal death.
Equity	Include women, communities, adolescents and affected groups in policy adaptation and service design.

Bottom Line

As of **29 April 2026**, the central WHO-based maternal-health guideline for diagnosis, treatment and management in this report is the **5 October 2025 WHO/FIGO/ICM consolidated PPH guideline**, which provides **51 actionable recommendations** and shifts care toward **objective blood-loss measurement, earlier diagnosis and rapid bundled treatment** [WHO PPH guideline](#), [NCBI Bookshelf executive summary](#). New evidence through the cutoff date reinforces—not overturns—WHO’s direction, especially for combining measured blood loss with haemodynamic signs and implementing structured PPH bundles. The **22 February 2026 WHO implementation guide** is the key post-guideline tool for translating these recommendations into national policy and clinical practice [WHO implementation guide](#).